

العمر(تاريخ الميلاد) : 05-05-1984 المهنة : ربه منزل

ملاحظات : -----

المجلس الطبي المتخصص

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Name: صفاء ابراهيم ابوالميط محمد National ID: 28405052101463	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
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المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Second Line Hormonal Treatment Plus Goserline
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



٢٠١٩/٧
٢٨٤٠٥٠٢١٠١٤٦٣
حاصلة على بطون صفين

التي
مستلمة
ممن
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٢٠٢٩/٧/٠١ حتى ٢٠٢٩/٧/٠١



ERM036744813



جواز سفر
ساعة
٢٨٤٠٥٠٢١٠١٤٦٣

التي
ممن
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٢٠٢٩/٧/٠١ حتى ٢٠٢٩/٧/٠١

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MDT FORM 21 / 1 / 2026 G

Name	هناد إبراهيم أبو حبيب	MRN	1771/C.50
Age	414.	ID	5050.00101273
Menopausal status	premen	Phone	01110488049
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity		REASON OF MDT	post NACT

Clinical Data

BX:	IDC II	R	ps:1	L
ER: +ve	PR: +ve	HER 2: -ve	Ki67: 35 %	
Sonomammography:				
premen Lgr Breast cancer				
CTub. N1, No CT & BS → for				
Mets Workup:				
Rec NACT				
4 AC, u Taxsteron 13/1/2026				
with good Regressive course				
Recommendations:				
for MDT for surgery				
د/سليم				
0-07/1/14				
By examination good response to NACT				
MDT OUTCOME & PLAN				
Still noted axillary LN				
Complete c-ct/1/14				
For LT MRN				

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Name: صفاء ابراهيم ابو الغيط محمد		
MR. : 1662		
Detailed Medical Report		
		السيد
تجبة طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشر بأن نوضح فيما يلي تقريرا عن حالتها		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انها The patient is a 41-year-old female with a history of left breast invasive ductal carcinoma, grade II, ER 6/8, PR 7/8, HER2-negative, Ki-67 73.5%, staged cT4b N2. She underwent neoadjuvant chemotherapy with 4 cycles of AC (doxorubicin/cyclophosphamide) followed by 4 cycles of docetaxel (TXT), achieving a pathological complete response (pCR) with free surgical margins and 0/13 lymph nodes positive. At multidisciplinary team review on 11/02/2026, adjuvant treatment was planned with a left chest wall and axillary lymph node irradiation (PORT) and adjuvant endocrine therapy consisting of Zoladex (goserelin) 10.8 mg SC and letrozole 2.5 mg daily for five years, with zoledronic acid q6 months for bone protection. The patient presented on 07/07/2026 with ECOG 1, reporting generalized bony aches. She completed a PORT course of 40 Gy in 15 fractions over 3 weeks, starting 30-04-2026 and ending 14-05-2026. She received Zoladex 10.8 mg and letrozole 2.5 mg, and supportive therapy for bone health. Dental consultation was scheduled for the next clinic visit to initiate zoledronic acid. The patient remains on the planned adjuvant endocrine therapy and bone-protective regimen.		
تحريرا في ,		
	الوقت :	التاريخ : 20-09-2026
	إيه مدحت بدوي محمد	الطبيب المعالج :
يعتمد ,		

Name : SAFAA IBRAHIM

Date : 13-01-2026 12:18

Branch : Portable

Medical No. : GR1-284050

Ref. Physician Prof :

Ins. Company :

LEFT BREAST HIGH RESOLUTION SONOGRAPHY REVEALED:

CLINICAL SUMMARY:

Left breast pathologically proven cancer, treated with NACT

FINDINGS:

LEFT BREAST:

- The large UOQ ill-defined solid mass is no more seen in today's study, only retroareolar area of altered parenchymal texture is seen associated with skin thickening, measuring totally about 3.5x2cm.
- The previously seen level I pathological node is no longer noted in today's study.
- Free infra and supra-clavicular regions

CONCLUSION:

Left breast (BIRADS 6): markedly regressive course of the malignant process.

Best regards,
Dr. Mahi Mokhtar

Patient Name	: الأستاذة / صفاء ابراهيم ابو الغيط	Request No.	: 13007031
Patient No.	: 28405052101463	Request Date	: 22/05/2025
Path No.	: 1860/25	Verification Date	: 04/06/2025
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 41 Y	Department	: Out Patient
Referring Doc :		Registered by	: rnarwa mohamed youss

Pathology Report

Left breast mass, Tru cut biopsy:

Invasive duct carcinoma, Grade II

IPT:

* ER: Positive (score 6/8).

* PR: Positive (score 7/8).

* HER2: Negative (score 0).

* Ki67: Positive in 35% of tumor cells.

Signature

Dr. Walaa Ibrahim Ghoneim

Dr. Heba Hassan

Verified by :

Dr. Heba Hasan

Printing Date : 11/06/2025 10:12:32

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Printing By : Mrs. Mayada Hegazy

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غرفة رقم (4) - الدور الاول - مبنى رقم (987) - بحورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadima - Cairo, Egypt



SCAN & LABS

مركز النيل

Name : Safaa Ibrahim Aboulghait

Date : 27-12-2025 12:12

Branch : Dokki

Medical No. : 282650

Ref.Physician Prof : Goustav Roussey Egypt

Ins.Company : تعاقد مستشفيات

Tc^{99m} MDPSKELETAL SCINTIGRAPHY

History

- A 41-year-old lady, with history of left breast cancer and referred for metastatic workup.

Technique

- Anterior and posterior whole-body scans were acquired 2 hours post injection of the tracer.

Findings

- No abnormal active or cold focal bony lesions that could account for metastatic bone disease.
- Degenerative changes are noted at the spine and big joints.

Impression:

- The skeleton is free from metastases.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

Local Center 19553



Patient Name: Safaa Ibrahim Aboul Ghait Mohamed	Date : 19/05/2025
Exam : MAMMOGRAPHY BOTH	Age : 41 Year(s)
Medical ID: 28405052101463	Pat. ID: 99697

DIGITAL BILATERAL SOFT TISSUE MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

Report:

- ACR b: scattered fibroglandular parenchyma
- Left Breast:
 - Highly suspicious Irregular dense mass is seen in the UOQ measuring 64 X 35 mm with overlying suspicious microcalcification cluster .
 - Ultrasound shows evidence of an irregular infiltrating suspicious hypoechoic mass lesion with overlying microcalcifications measures about 60 x 30 mm located from 12 clock to 3 clock with Retroareolar extension reaching to the nipple BIRADS 5 lesion for histopathological correlation
 - One pathologically enlarged left axillary lymph nodes level 1 axillary group 12 x 8 mm with 5 mm cortex and eccentric reduced fatty hilum. .
 - No supra or infraclavicular lymph nodes.
- Right Breast:
 - No suspicious micro calcifications or architectural distortion detected.
 - Complementary ultrasound no cystic or solid mass lesions.
 - Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
 - Nonspecific axillary LN.

Opinion:

- Left breast show a highly suspicious mass lesion malignant looking , as described above (BIRADS 5), for histopathological correlation .
- Normal sonomammography of right breast (BIRADS 1)

Signature Dr . Rehab El Badry

19/05/2025 2:25 PM

Patient Name : الأستاذ / صفاء ابراهيم ابو الغيط	Request No. : 13007031
Patient No. : 28405052101463	Request Date : 22/05/2025
Path No. : 1860/25	Verification Date : 04/06/2025
Sex : Female	Financial Category : اورام دار السلام - مبادره
Age : 41 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical Data

Left breast mass

Nature of specimen

Tru cut biopsy, collected at 22/5/2025, 10:30 am

Gross examination

Multiple grayish white tissue cores up to 1.5 cm, totally submitted.

IHC technique: Sections prepared from paraffin block for immune-staining using Bench

Mark ULTRA system for ER, PR, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-3).

Microscopic examination

- * Histological type: Invasive duct carcinoma
- * Grade (differentiation +nuclear pleomorphism+ mitosis): (3+2+2) Grade II
- * DCIS: No
- * Angiolinvasion: Couldn't be detected in the received biopsy
- * Perineural invasion: No
- * IPT (Positive control showed appropriate reactivity) :
- * E-cadherin : Diffuse positive
- * ER: Strong nuclear reaction in 20 % tumor cells (Allred score 6/8)
- * PR: Strong nuclear reaction in 50% tumor cells (Allred score 7/8)
- * HER2: No membrane reaction in of tumor cells (score 0)
- * Ki 67: 35 % nuclear reaction of tumor cells

Diagnosis

Verified by :

Dr.Heba Hasan

Printing Date : 11/06/2025 10:12:32

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